CONSULT FOR AFRICA
Build. Transform. Grow.

PREPARED FOR

Mrs Fola Laoye

Chief Executive Officer
Iwosan Investments / IASO Medipark

AN INTRODUCTION / OCTOBER 2026

Healthcare advisory **that carries the operating result**

We build, commission, fix and run health businesses in Nigeria, and we take build and operate mandates as well as advisory ones, so the plans we write come back to us as a number we have to hit.

N1.7bn
medical campus structured and costed, twelve businesses in occupation

6 weeks
from nothing to live for a 90-bed field hospital and its intensive care unit

10,000+
verified Nigerian clinicians on a registry we build and maintain ourselves

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WHERE THIS COMES FROM

Sixteen years of running hospitals

Consult for Africa came out of the operating jobs, not out of a consulting firm.

2019

Doctoora

A technology-enabled private healthcare network built from nothing across 21 states, and sold to Evercare

2020

Eti-Osa

The COVID isolation centre. 90 beds, a 10-bed ICU and the state reporting dashboard, live in six weeks

2021

Evercare

Chief Innovation and Strategy Officer. USD 1.1m of cost out with volume held, and pay for performance in

2024

Cedarcrest

Chief Executive. N1.0bn a month and N1bn H1 EBITDA, the strongest year in the hospital's history

NOW

Consult for Africa

The same work, done for owners and boards rather than for one employer, and increasingly with our own money in it

Everything the firm sells, somebody here has already had to do with their own name against the number at the end of the month.

HOW WE WORK

Four ways clients use us

Consult for Africa is a healthcare advisory and venture firm in Lagos and Abuja, partner led and built by operators. Nobody should buy the fourth of these before they have seen us do the first.

01

Diagnostic

Two to four weeks

A fixed scope. We count the thing, trace the cases and come back with what is actually true. Most clients start here and some stop here.

02

Advisory

Eight to sixteen weeks

Strategy, feasibility, financial model, operating design or governance build, delivered as instruments the team can use rather than as recommendations.

03

Build and commission

Three to twelve months

We stand the thing up. Licensing, fit-out sequencing, staffing, tariffs, systems and the opening plan, then we hand over a running business.

04

Build and operate

Two to five years

We install and hold the management team and carry the number. The incentive is subordinated to the owner's return of capital: a lower share until the money is home, a higher one after.

Advisory work is billed as resourced time: named grades at published rates against the days each piece actually takes, invoiced monthly, so nothing you drop is ever paid for.

The space that is not the hospital needs its own owner

Otherwise it sits empty for eighteen months while carrying its share of the fixed cost, because the hospital takes every hour of management attention in the opening year.

- **Measured geometry, not the brief.** We measure what is actually built, because gross to net is where campus models break and the error is always in the corridors.
- **A rent roll and an occupancy model.** Every non-clinical line given a tenant or an operator, a price, an occupancy target and a named owner before opening.
- **A monthly cash model.** Peak funding, the trough month and the payback, modelled monthly, because annual models hide the months that decide whether the thing is fundable.
- **Tenant mix.** Which occupiers feed the hospital, which compete with it, and what each one should be paying for the footfall.
- **An operating model per line.** Diagnostics, training, retail and residential each run as a business with its own profit and loss.

WHERE WE HAVE DONE IT

N1.7bn

medical campus structured and costed last year: twelve businesses in occupation, measured geometry, rent roll, monthly cash model, peak funding and a 29-month payback.

On that campus roughly a third of stabilised revenue came from lines that were not the anchor clinical service, and the fastest of them was paying before the anchor was.

A hospital earns its first year in the ninety days after it opens

So we plan that period as an operating period, and we stay in the building for it.

- **Readiness and sequencing.** A service-by-service opening plan with readiness gates, so the first patients arrive in the lines that are genuinely ready.
- **The ramp, modelled monthly.** Volume, revenue, staffing and cash by month across the first year, with one person accountable to the number rather than to the date.
- **Licensing and regulatory.** HEFAMAA, scope of practice, NAFDAC where relevant, and the inspection file assembled before anybody asks for it.
- **Systems before day one.** Hospital information system, tariff schedule, rotas, standard operating procedures and the reporting cadence, so month one produces management information rather than anecdote.
- **Day one to day ninety.** The period that decides the first year, worked through with the team rather than reviewed afterwards.

WHERE WE HAVE DONE IT

6 weeks

from nothing to live for a 90-bed field hospital with a 10-bed intensive care unit, a customised health information system and the state-wide reporting dashboard.

Eti-Osa, 2020, for the Lagos State response. Licensing, fit-out sequencing, staffing, systems and the opening plan, all of it compressed.

Lagoon, Euracare and Medipark, run as one group

The stated model is that each hospital keeps its identity and benefits from shared resources underneath. The work is settling what actually gets shared, and what each address is for when several of them sit minutes apart.

- **What sits where.** Service configuration across the brands, so two of your own addresses are not competing for the same consultant and the same patient.
- **Referral architecture.** How patients move between addresses, who owns the profit and loss when either could treat them, and how the referral is rewarded.
- **Shared services.** What consolidates, what stays local, and who backfills the seats a new site empties when it takes your best people.
- **Fractional and seconded leadership.** Chief executive, operating, medical and technology cover on fixed-term mandates, sitting inside the building rather than reporting into it.
- **Build and operate.** Where an owner wants it, we install and hold the management team and carry the number, with our incentive subordinated to their return of capital.

WHERE WE HAVE DONE IT

USD 1.1m

of annualised cost taken out of a 165-bed fund-owned tertiary hospital through workforce and service portfolio rebalancing, with clinical volume held.

Elsewhere, a leadership structure taken from twenty roles to eight, and an asset-light expansion model under which a hospital takes over management of existing facilities with no capital expenditure.

Tariff and payer mix, before volume

Especially where a concession has already fixed part of the bed base and the rest of it has to carry the cost.

- **Tariff architecture.** Price positions by site and by service line, benchmarked, with an explicit written position on what each address is for.
- **Payer strategy.** Managed care terms and where to refuse them, corporate and embassy contracting, and the self-pay proposition said in a sentence that is not about equipment.
- **Revenue integrity.** Billing capture, claims recovery, payer reconciliation and the leakage controls that stop the same money going missing twice.
- **Packaged pathways.** Priced end to end, consultant named, escalation written down. This is what converts a family that would otherwise travel, because they are buying certainty rather than capability.
- **Service line economics.** Contribution by line, so the mix is managed deliberately rather than inherited.

WHERE WE HAVE DONE IT

N1.0bn

a month at a 75-bed Abuja tertiary surgical hospital, the strongest year in its sixteen-year history, built on tariff realignment, service mix and revenue governance.

N1bn of EBITDA in the first half, ahead of the whole of the prior year, with no new capacity added.

Hiring a thousand people to a date is an operations problem

Knowing good clinicians and being able to seat a thousand of them against an opening date are different problems. We are built for the second one.

- **Named-seat planning.** The consultant bench mapped specialty by specialty against the service lines that carry margin, with lead times counted backwards from the opening date.
- **Volume without dropping the bar.** Sourcing, screening and offer management run as a pipeline with a weekly fill rate against each seat, including returners where the role suits one.
- **Structured assessment.** Credential verification and a structured leadership assessment on every senior appointment, so a board appoints on evidence rather than on interview performance.
- **Pay that survives year two.** Fixed and variable design, session economics, and the governance that keeps it honest once volumes move.
- **Locum and interim cover.** Temporary clinical cover filled in days, which is what keeps a new rota standing through its first six months.

WHERE WE HAVE DONE IT

10,000+

verified Nigerian clinicians across sixteen cadres, on a registry we build and maintain ourselves, with salary and supply benchmarked by cadre and by state.

We also designed a consultant pay-for-performance model that lifted revenue 23 per cent in three months while raising variable pay 15 per cent.

How a first piece of work gets scoped

We do not ask anybody to commission a programme on the strength of a profile. The first piece of work is deliberately small, so that the decision to carry on is made on something we have actually done for you rather than on something we have written about ourselves.

- **A conversation.** Twenty minutes, to work out which of the six is genuinely heavy and which is already handled.
- **A scope.** One page, fixed, with a deliverable and a date on it. Priced once we have agreed what is in it, which is a conversation rather than a document.
- **Two or three weeks.** We do the work, inside the building, with whoever owns it on your side.
- **Then you decide.** Either there is something worth carrying on with, or you have an early and inexpensive answer. In our experience both are worth having.

ON FEES

We would rather show you the method than quote you a figure in a document nobody asked for. Beyond a fixed first piece, advisory work is billed as resourced time: named grades at published rates against the days each piece actually takes, invoiced monthly.

On build and operate mandates the incentive is subordinated to the owner's return of capital. A lower share until the money is home, a higher one after.

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THE ASK

Twenty minutes, whenever suits. **And I would start with the campus.**

A medipark is a property business with a hospital inside it, and the diagnostics, training, retail and residential each need an operator and an occupancy number of their own. It is the piece least like anything else in the group. If it is already settled, say so and I will ask about the ramp instead, and if the timing is wrong altogether, tell me and I will come back after you have opened.

Either way, I would like to see the place before it opens.



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